

Pain Scale Chart

A steadier 0 to 10 anchor for people who need the number to mean the same thing tomorrow that it meant today.

Routing Note

Need: Pain scores collapse when the scale keeps moving. This chart gives the number a spine again.
Tool: Use it beside any log when you need your ratings to stay coherent across bad days, better days, and appointments.
Rule: Rate from function and lived reality. Not from shame. Not from performance. Not from what sounds respectable.

1. Numeric Rating Scale (NRS) 0-10

Use the functional descriptions below to rate your pain -- match your experience to the description, not just the number.

0	No Pain No pain sensation at all. Completely comfortable.	NORMAL
1	Minimal Barely noticeable. Easily ignored. Does not interfere with any activities.	NORMAL
2	Mild Minor pain, noticeable but not distracting. Activities continue without effort.	MILD
3	Uncomfortable Noticeable and can be distracting. Manageable with effort. Able to adapt and work.	MILD
4	Moderate Noticeable most of the time. Can be ignored with effort. Some activities affected.	MODERATE
5	Moderately Severe Cannot be ignored for more than a few minutes. Activity is limited.	MODERATE
6	Severe Pain dominates thinking. Concentration is difficult. Significantly limits function.	MODERATE
7	Very Severe Hard to function normally. Interferes with sleep and conversation. Hard to concentrate.	SEVERE
8	Intense Physical activity severely limited. Difficulty speaking or moving. Overwhelming.	SEVERE
9	Excruciating Unable to function. Crying, moaning, or unable to speak. Near worst imaginable.	SEVERE
10	Worst Possible Worst pain imaginable. Completely incapacitating. Emergency level.	EMERGENCY

2. Quick-Reference Color Strip

Cut out or photocopy this strip. Keep it with your pain diary for fast, consistent rating.



3. Functional Impact by Category

Pain affects function differently at each level. Use this table to rate based on what you CAN and CANNOT do.

Category	Range	Activity	Sleep	Work	Mood
Normal	0-1	Fully active	No impact	Full capacity	Unaffected
Mild	2-3	Active with awareness	Falls asleep OK	Full with effort	Slight frustration
Moderate	4-6	Limited activities	Disturbed sleep	Reduced capacity	Anxious, irritable
Severe	7-8	Basic self-care only	Poor or no sleep	Unable to work	Distressed
Crisis	9-10	Cannot move or function	Cannot sleep	Incapacitated	Overwhelmed

4. Common Rating Mistakes

Mistake	Why It Matters	Instead, Do This
Comparing to worst-ever pain	Makes current pain seem less important	Rate based on today's function, not past extremes
Comparing to other people	Everyone's scale is personal	Match YOUR experience to the descriptions above
Rating from memory hours later	Memory exaggerates peaks, forgets averages	Rate in the moment or as soon as possible
Always picking a round number	Misses gradual changes over time	Use odd numbers too -- 3 and 5 are different
Minimizing to seem tough	Under-reporting delays treatment changes	Honest rating helps your provider help you
Only rating bad days	Missing good days distorts averages	Track every day including low-pain days

5. Pain Quality Descriptors

When describing pain to your doctor, the NUMBER tells them how much. These WORDS tell them what kind.

Type	Feels Like	Common In	Example
Sharp / Stabbing	Knife-like, sudden, piercing	Nerve injury, acute injury	Sciatica, herniated disc
Dull / Aching	Deep, constant, heavy pressure	Muscle strain, arthritis	Low back pain, joint aches
Burning	Hot, stinging, surface-level	Nerve damage, inflammation	Neuropathy, sunburn-like pain
Throbbing	Pulsating, rhythmic, pounding	Vascular, infection, migraine	Headaches, dental pain
Cramping	Squeezing, tightening, spasm	Muscle spasm, visceral	Menstrual cramps, GI pain
Tingling / Numb	Pins and needles, loss of sensation	Nerve compression	Carpal tunnel, pinched nerve
Radiating	Spreading outward from a point	Nerve root, referred pain	Sciatica down the leg

6. When to Seek Medical Attention

Use this guide alongside your pain rating. Some situations need prompt attention regardless of the number.

Situation	Action	Why
Pain 8-10 that does not improve	Seek urgent care or ER	Uncontrolled severe pain needs medical management
Sudden new pain with numbness or weakness	Call 911 or go to ER	May indicate nerve damage or stroke
Pain after injury with swelling or deformity	Go to ER or urgent care	Possible fracture or serious tissue damage
Pain with fever and chills	See doctor same day	May indicate infection needing antibiotics
Pain steadily increasing over days/weeks	Schedule appointment this week	Progressive pain suggests worsening condition
Pain unchanged despite treatment	Follow up with provider	Treatment plan may need adjustment

7. How to Describe Pain to Your Doctor

Use this framework at appointments. Providers need specific detail to diagnose and treat effectively.

LOCATION: Where exactly does it hurt? Does it spread anywhere?

ONSET: When did it start? What were you doing?

CHARACTER: What does it feel like? (See Pain Quality Descriptors above)

RATING: Use 0-10. Give your current, average, and worst this week.

TIMING: Is it constant or intermittent? Worse at certain times?

AGGRAVATING: What makes it worse? (movement, stress, weather, position)

RELIEVING: What helps? (rest, meds, heat/ice, position changes)

SEVERITY: How does it affect daily life? (sleep, work, mood, self-care)

8. Rules for a Stable Scale

1. Rate close to the moment. The number drifts fast once memory starts rewriting the scene.
2. Use function as the anchor. A 6 is not decoration. It means something about what the body can still do.
3. Keep the better days. Without them, the averages lie.
4. Tie each number to context: what made it rise, what brought it down, what it cost.
5. Bring the same scale into appointments so the conversation starts from shared ground.
6. Fluctuation is not failure. Rate the real period you are talking about, not the most cinematic moment in it.
7. Your scale is yours. The job is consistency, not performance for someone else.

Why the scale has to hold:

If the scale keeps shifting, treatment response disappears into noise. A stable number is not paperwork.
It is what makes change visible. | paintracker.ca